

ORIGINAL RESEARCH

HEART FAILURE

Finerenone-Related Risk of Hypotension in Heart Failure With Mildly Reduced or Preserved Ejection Fraction



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ABSTRACT

BACKGROUND The nonsteroidal mineralocorticoid receptor antagonist finerenone reduces clinical events in heart failure with mildly reduced ejection fraction/preserved ejection fraction; however, the implications of treatment-related hypotension are unknown.

OBJECTIVES The authors investigated predictors of systolic blood pressure (SBP) <100 mm Hg and investigator-reported hypotension and their associations with randomized treatment and clinical outcomes in the FINEARTS-HF (Study to Evaluate the Efficacy [Effect on Disease] and Safety of Finerenone in Participants With Heart Failure and Left Ventricular Ejection Fraction [Proportion of Blood Expelled Per Heart Stroke]) trial.

METHODS FINEARTS-HF was a randomized, placebo-controlled trial of finerenone in symptomatic patients with chronic heart failure (left ventricular ejection fraction $\geq 40\%$). Predictors of SBP <100 mm Hg and hypotension were identified using Cox models. Associations between SBP <100 mm Hg and hypotension, treatment, and clinical outcomes were evaluated using time-updated Cox models. The primary outcome was a composite of total heart failure events and cardiovascular death.

RESULTS Among the 5,815 participants with available data, post-baseline SBP <100 mm Hg occurred in 899 (538 with finerenone vs 361 with placebo; odds ratio: 1.60; 95% CI: 1.38-1.85) and investigator-reported hypotension in 364 patients (225 with finerenone vs 139 with placebo; odds ratio: 1.67; 95% CI: 1.34-2.08). Participants experiencing SBP <100 mm Hg had lower baseline SBP, were older, had higher N-terminal pro-B-type natriuretic peptide levels, a history of smoking, and no diabetes. Treatment-related risk of the primary endpoint was reduced in patients with no/before SBP <100 mm Hg (rate ratio: 0.78; 95% CI: 0.67-0.90) and appeared to attenuate afterwards (rate ratio: 0.99; 95% CI: 0.70-1.39), although no formal statistical interaction was observed ($P_{\text{interaction}} = 0.33$).

CONCLUSIONS In this prespecified analysis of the FINEARTS-HF trial, finerenone led to higher rates of post-baseline SBP <100 mm Hg and investigator-reported hypotension. Although hypotension should not prompt automatic treatment discontinuation, these patients should be carefully monitored. (Study to Evaluate the Efficacy (Effect on Disease) and Safety of Finerenone in Participants With Heart Failure and Left Ventricular Ejection Fraction (Proportion of Blood Expelled Per Heart Stroke) Greater or Equal to 40% [FINEARTS-HF]; [NCT04435626](https://doi.org/10.1016/j.jchf.2025.102779)) (JACC Heart Fail. 2026;14:102779)
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ABBREVIATIONS AND ACRONYMS

ARNI = angiotensin-receptor neprilysin inhibitor

COPD = chronic obstructive pulmonary disease

eGFR = estimated glomerular filtration rate

HFrEF = heart failure with reduced ejection fraction

HFmrEF = heart failure with mildly reduced ejection fraction

HFpEF = heart failure with preserved ejection fraction

MRA = mineralocorticoid receptor antagonist

NT-proBNP = N-terminal pro-B-type natriuretic peptide

SGLT2 = sodium-glucose cotransporter 2

The current management of patients with heart failure (HF) relies on the initiation of multiple hemodynamically active therapies, including those that antagonize the renin-angiotensin-aldosterone system.^{1,2} However, these guideline-directed medical therapies often lower blood pressure and increase the risk of hypotension, thereby limiting treatment initiation or dose up titration in clinical practice. Importantly, the occurrence of hypotension has been associated with increased risk of subsequent fatal and nonfatal events across the full left ventricular ejection fraction (LVEF) spectrum.³⁻⁵

Over the past decades, steroidal mineralocorticoid receptor antagonists (MRAs) have been shown to improve clinical outcomes in heart failure with reduced ejection fraction (HFrEF) and in patients with acute myocardial

infarction complicated by reduced LVEF.^{6,7} The nonsteroidal MRA finerenone was shown to slow kidney disease progression and prevent cardiovascular (CV) events in patients with chronic kidney diseases and type 2 diabetes^{8,9} and has recently been approved for the management of HF with an LVEF $\geq 40\%$ by the U.S. Food and Drug Administration.¹⁰ In the FINEARTS-HF (Finerenone Trial to Investigate Efficacy and Safety Superior to Placebo in Patients with Heart Failure) trial, finerenone reduced the risk of the primary composite outcome of total HF events and CV death in patients with heart failure with mildly reduced ejection fraction (HFmrEF) or heart failure with preserved ejection fraction (HFpEF).¹¹ Antagonists of the mineralocorticoid receptor pathway, including finerenone, are known to lower blood pressure. In this prespecified analysis of the FINEARTS-HF trial, we investigated the predictors and prognostic impact of hypotension and the associations between treatment,

hypotension, and clinical outcomes in patients with HFmrEF/HFpEF.

METHODS

The study design and the primary results of the FINEARTS-HF trial have been previously described.^{11,12} The trial protocol was approved by the ethics committee at all participating institutions, and all participants provided written informed consent. Briefly, FINEARTS-HF was an international, multicenter, randomized, double-blind, placebo-controlled clinical trial testing the efficacy and safety of finerenone in patients with symptomatic HFmrEF/HFpEF. Key inclusion criteria were ≥ 40 years of age, NYHA functional class II to IV, diuretic treatment within 30 days before randomization, an LVEF $\geq 40\%$, evidence of structural heart disease, and elevated natriuretic peptides with different cutoffs based on atrial fibrillation status. Key exclusion criteria included: estimated glomerular filtration rate (eGFR) < 25 mL/min/1.73 m², potassium levels > 5.0 mmol/L, recent myocardial infarction, coronary artery bypass grafting or percutaneous coronary intervention, and history of cardiomyopathies or inflammatory heart disease. Additionally, patients with symptomatic hypotension with mean systolic blood pressure (SBP) < 90 mm Hg at screening or at randomization were excluded. Eligible patients were randomized in a double-blind 1:1 fashion to either finerenone or matching placebo. Participants with an eGFR ≤ 60 mL/min/1.73 m² started with 10 mg once daily with a maximum maintenance dose of 20 mg, whereas participants with an eGFR > 60 mL/min/1.73 m² started with 20 mg with a maximum maintenance dose of 40 mg.

HYPOTENSION DEFINITIONS. Among participants who received at least 1 dose of finerenone or placebo,

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2 definitions of post-randomization treatment-emergent hypotension were included in the study protocol, both occurring either during treatment or within 3 days of permanent discontinuation: 1) measurement of SBP <100 mm Hg at any scheduled study visit, regardless of symptoms (specifically, 2 consecutive blood pressure measurements were taken at least 2 minutes apart in sitting position at every study visit and the average was calculated for the statistical analysis); measurements were assessed with a completely automated device and preceded by at least 10 minutes of rest for the participant in a quiet setting without distractions (eg, television, cell phones); and 2) investigator-reported hypotension not associated to a specific SBP value.

Only participants with available data on follow-up SBP measurements were included in this analysis.

CLINICAL OUTCOMES. The primary outcome of the FINEARTS-HF trial was a composite of total (first and recurrent) HF events (HF hospitalization or HF urgent visit) and CV death. Other outcomes included in this analysis were the individual components of the primary endpoint, all-cause mortality, CV hospitalizations, and all-cause hospitalizations.

STATISTICAL ANALYSIS. Analyses were performed based on the first occurrence of SBP <100 mm Hg and investigator-reported hypotension. Risk of SBP <100 mm Hg and investigator-reported hypotension was expressed with odds ratios. Baseline characteristics were summarized as mean $\pm$ SD, median (Q1-Q3), or frequencies (%). Baseline characteristics were compared between participants who did and did not develop SBP <100 mm Hg and investigator-reported hypotension using Student's *t*-tests or Wilcoxon rank-sum tests for comparison of continuous variables, as appropriate, and chi square tests for comparison of categorical variables.

Predictors of hypotension were assessed using multivariable Cox models using a stepwise forward selection adding variables with a value of $P < 0.05$.

The association between baseline SBP as a continuous variable, treatment, and incidence rates of SBP <100 mm Hg and investigator-reported hypotension was assessed with Poisson regression models using restricted cubic splines.

Associations between both SBP <100 mm Hg and investigator-reported hypotension and clinical outcomes were evaluated using time-updated Cox proportional hazard models with and without adjustment for age, region, sex, randomized treatment, LVEF, baseline SBP, heart rate, cigarette smoking, type 2 diabetes, eGFR, body mass index, beta-blockers, angiotensin-converting enzyme inhibitor/

angiotensin receptor blocker/angiotensin receptor neprilysin inhibitor (ARNI), sodium-glucose cotransporter 2 (SGLT2) inhibitor, atrial fibrillation, and log-transformed N-terminal pro-B-type natriuretic peptide (NT-proBNP). Event rates were estimated as the number of events over the total duration of follow-up and expressed as a rate per 100 patient-years. For the analyses of total events, the semiparametric method of Lin et al¹³ was used and risk expressed as rate ratio (RR) rather than HR.

To characterize the safety and tolerability of finerenone after a hypotensive event, all event rates and treatment effects were evaluated in both an intention-to-treat (ITT) analysis and among participants while adherent to randomized treatment (on-treatment analysis). The associations between hypotension, treatment, and clinical outcomes were evaluated using time-updated Cox models with adjustment for a propensity score (using the predictors previously identified) representing the propensity to develop post-randomization SBP <100 mm Hg and hypotension and were stratified for region and LVEF.

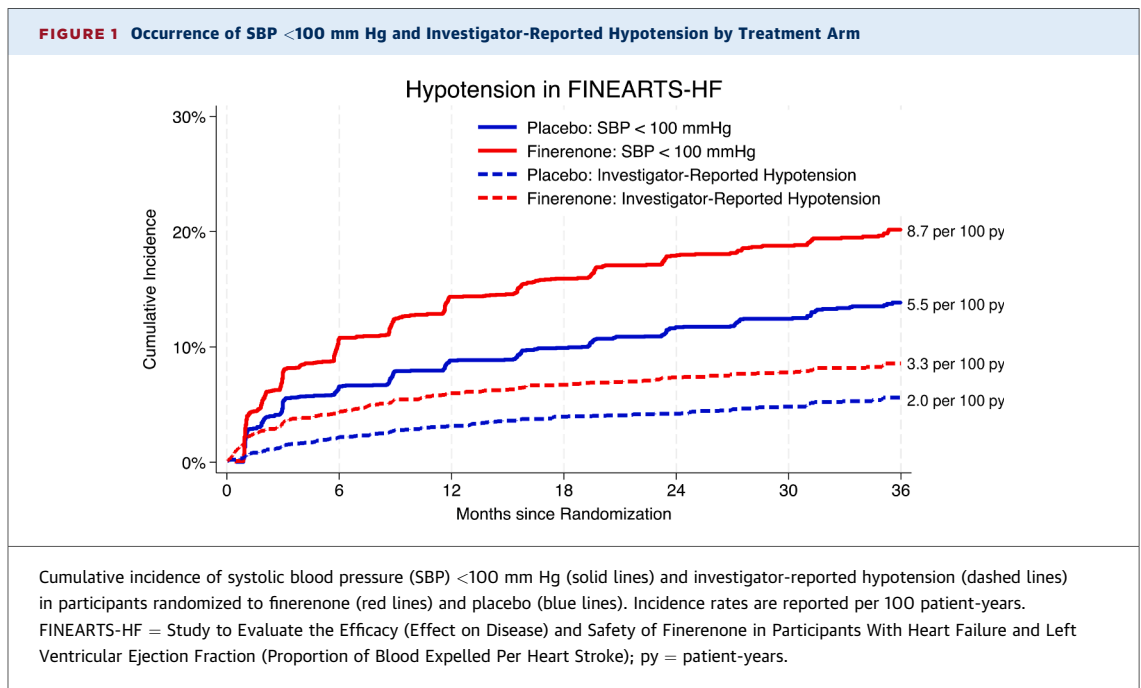
Time on study drug was calculated from randomization to the date of last recorded dose, plus up to a 3-day "grace period." Permanent study drug discontinuation was defined as occurring when the last day of treatment exposure preceded death or censoring.

Statistical analyses were performed using STATA (version 18, StataCorp). Values of $P < 0.05$ were considered statistically significant.

RESULTS

BASELINE CHARACTERISTICS. Among 6,001 participants in the FINEARTS-HF trial, data regarding post-baseline SBP measurements were missing in 186 patients who were excluded, leaving 5,815 participants who were analyzed in this study. Randomization to finerenone was well balanced among patients with available data (50.1%) and those without post-baseline SBP measurements (49.5%). Baseline characteristics in patients with available post-baseline SBP vs those with missing values are summarized in [Supplemental Table 1](#).

At 3-months post-randomization, finerenone lowered the mean SBP by 3.1 mm Hg (95% CI: 2.5-3.7 mm Hg). Over a median follow-up of 32 months, 899 of the 5,815 (15.5%) patients included in the analysis experienced SBP <100 mm Hg, more frequently in the finerenone study arm (538 [18.5%] vs 361 [12.4%]; OR: 1.60 [95% CI: 1.38-1.85]; $P < 0.001$). The cumulative incidence of post randomization SBP <100 mm Hg gradually accrued over time and fewer participants (364 [6.3%]) experienced investigator-reported



hypotension, which was more frequent among people randomized to finerenone (225 [7.7%] vs 139 [4.8%]; OR: 1.67; 95% CI: 1.34-2.08; $P < 0.001$) (Figure 1). Investigator-reported hypotension occurred in 153 of 899 (17%) people with SBP <100 mm Hg, and hypotension-related hospitalizations occurred in 20 participants, without statistically significant differences between study arms.

Patients' baseline characteristics according to SBP <100 mm Hg occurrence are summarized in Table 1. Participants experiencing SBP <100 mm Hg were older, more often male, less likely to be White and more frequently Asian. These patients had a lower baseline SBP (118 vs 132 mm Hg), were less likely to have a history of diabetes or atrial fibrillation, had a lower eGFR, and presented with higher NT-proBNP levels. Patients who developed SBP <100 mm Hg were less frequently receiving antihypertensive medications and were more likely on SGLT2 inhibitors and loop diuretics.

Participants' baseline characteristics according to investigator-reported hypotension are summarized in Supplemental Table 2.

PREDICTORS OF HYPOTENSION. Patients' baseline characteristics independently associated with SBP <100 mm Hg occurrence were lower SBP at randomization, Asian race, randomization to finerenone, higher NT-proBNP levels, older age, greater body mass index, cigarette smoking, lack of diabetes

history, a longer time from a HF event or no HF event, concomitant use of SGLT2 inhibitors and loop diuretics, and no background use of calcium channel blockers (Table 2).

Independent predictors of investigator-reported hypotension are shown in Supplemental Table 3.

Baseline SBP was a strong predictor of both SBP <100 mm Hg and investigator-reported hypotension with incidence rates increasing for values <120 mm Hg without significant differences between study arms (Figure 2).

No significant interaction was observed between finerenone target dose and risk of SBP <100 mm Hg ($P_{\text{interaction}} = 0.16$) or investigator-reported hypotension ($P_{\text{interaction}} = 0.35$).

OUTCOMES FOLLOWING HYPOTENSION. Subsequent clinical outcomes based on SBP <100 mm Hg occurrence are summarized in Table 3. The rates of the primary outcome of CV death and total HF events increased from 15.2 per 100 patient-years in participants not experiencing/before SBP <100 mm Hg to 23.4 per 100 patient-years following SBP <100 mm Hg, with a comparable risk after covariate adjustment. Following SBP <100 mm Hg, patients had an increased subsequent risk for CV death (adjusted HR [aHR]: 1.37 [95% CI: 1.04-1.81]; $P = 0.026$), and all-cause mortality (aHR: 1.38 [95% CI: 1.13-1.68]; $P = 0.001$). The risk for HF events was similar based on SBP <100 mm Hg

occurrence while all-cause hospitalizations were more frequent following SBP <100 mm Hg (aHR: 1.28 [95% CI: 1.08-1.42]; $P = 0.003$).

Despite the lower number of events, results appeared consistent when analyzing clinical outcomes according to investigator-reported hypotension (Supplemental Table 4).

ASSOCIATION BETWEEN TREATMENT WITH FINERENONE, HYPOTENSION, AND SUBSEQUENT RISK. In the primary ITT analysis, finerenone-related benefits on the primary outcome appeared more evident in participants not experiencing/before SBP <100 mm Hg (RR: 0.78; 95% CI: 0.67-0.90) and attenuated after SBP <100 mm Hg (RR: 0.99; 95% CI: 0.70-1.39); however, no significant interaction was observed ($P_{\text{interaction}} = 0.33$). Similar findings were observed for HF events (Table 4).

After SBP <100 mm Hg, the incident rates of permanent study drug discontinuation increased from 12.9 to 20.8 per 100 patient-years, with no differences between study arms. In the 30 days following SBP <100 mm Hg, an estimated 4% of patients permanently discontinued study drug (5% of finerenone patients, 3% of placebo patients) but not necessarily due to hypotension.

In the on-treatment analysis among participants adherent to randomized treatment, the benefits of finerenone on the primary outcome appeared to be maintained in people before/without SBP <100 mm Hg (RR: 0.66; 95% CI: 0.57-0.77) and after SBP <100 mm Hg occurrence (RR: 0.62 [95% CI: 0.42-0.90]; $P_{\text{interaction}} = 0.45$). Similar findings were observed when analyzing total HF events (Supplemental Table 5).

Following investigator-reported hypotension, the incidence rates of permanent study drug discontinuation increased to 22.3 per 100 patient-years, with no differences between study arms. In the 30 days following hypotension, an estimated 7% of patients permanently discontinued study drug (7% of finerenone patients, 8% of placebo patients). The association between treatment with finerenone, investigator-reported hypotension, and subsequent risk of the primary outcome is presented in Supplemental Table 6 (ITT analysis) and in Supplemental Table 7 (on-treatment analysis).

DISCUSSION

In this prespecified analysis of the FINEARTS-HF trial, the occurrence SBP <100 mm Hg was common during follow-up, occurring in more than 1 in 7 participants, whereas investigator-reported hypotension

TABLE 1 Baseline Characteristics According to Post-Baseline Treatment-Emergent SBP <100 mm Hg

	No SBP <100 mm Hg (n = 4,916)	SBP <100 mm Hg (n = 899)	P Value
Age, y	71.8 ± 9.6	72.6 ± 10.0	0.018
Female	2,262 (46.0)	363 (40.4)	0.002
Race			<0.001
White	4,068 (82.8)	516 (57.4)	
Black	75 (1.5)	11 (1.2)	
Asian	618 (12.6)	349 (38.8)	
Other	155 (3.2)	23 (2.6)	
Region			<0.001
Asia	610 (12.4)	344 (38.3)	
Eastern Europe	2,436 (49.6)	143 (15.9)	
Latin America	536 (10.9)	89 (9.9)	
North America	339 (6.9)	114 (12.7)	
Western Europe	995 (20.2)	209 (23.2)	
Prior HF hospitalization	2,939 (59.8)	544 (60.5)	0.68
Time since HF event			<0.001
≤7 d	1,034 (21.0)	125 (13.9)	
>7 d to 3 mo	1,621 (33.0)	343 (38.2)	
>3 mo or no index event	2,261 (46.0)	431 (47.9)	
Hypertension	4,475 (91.0)	684 (76.1)	<0.001
Type 2 diabetes	2,048 (41.7)	322 (35.8)	<0.001
Cigarette smoking			<0.001
Never	3,099 (63.0)	480 (53.4)	
Former	1,397 (28.4)	342 (38.0)	
Current	420 (8.5)	77 (8.6)	
Previous myocardial infarction	1,287 (26.2)	222 (24.7)	0.35
Coronary artery disease	1,910 (38.9)	326 (36.3)	0.14
Previous stroke	662 (13.5)	137 (15.2)	0.16
COPD	630 (12.8)	118 (13.1)	0.80
Atrial fibrillation	3,115 (63.4)	487 (54.2)	<0.001
History of LVEF <40%	192 (3.9)	72 (8.0)	<0.001
Systolic blood pressure, mm Hg	131.6 ± 14.2	117.6 ± 15.6	<0.001
Diastolic blood pressure, mm Hg	76.3 ± 10.0	71.1 ± 10.8	<0.001
Heart rate, beats/min	71.3 ± 11.6	72.1 ± 12.7	0.07
BMI, kg/m ²	30.2 ± 6.0	28.4 ± 6.4	<0.001
eGFR, mL/min/1.73 m ²	62.5 ± 19.7	60.7 ± 19.7	0.010
Creatinine, mg/dL	1.13 ± 0.4	1.14 ± 0.3	0.24
LVEF, %			0.044
<50%	1,756 (35.8)	350 (39.0)	
≥50% and <60%	2,193 (44.7)	394 (43.9)	
≥60%	961 (19.6)	154 (17.1)	
NYHA functional class			0.61
II	3,399 (69.2)	636 (70.7)	
III	1,486 (30.2)	257 (28.6)	
IV	30 (0.6)	6 (0.7)	

Continued on the next page

was less frequent. Both SBP <100 mm Hg and investigator-reported hypotension were more common in participants randomized to finerenone. Following SBP <100 mm Hg, patients faced a higher risk of mortality (both all-cause and CV death), whereas the risk of HF events was comparable. Although study drug discontinuation increased

TABLE 1 Continued

	No SBP <100 mm Hg (n = 4,916)	SBP <100 mm Hg (n = 899)	P Value
Beta-blockers	4,172 (84.9)	778 (86.5)	0.19
ACEI	1,852 (37.7)	245 (27.3)	<0.001
ARB	1,771 (36.0)	273 (30.4)	0.001
ARNI	330 (6.7)	163 (18.1)	<0.001
ACEI/ARB/ARNI	3,942 (80.2)	681 (75.8)	0.002
SGLT2 inhibitors	625 (12.7)	173 (19.2)	<0.001
Calcium channel blockers	1,718 (34.9)	197 (21.9)	<0.001
Loop diuretics	4,247 (86.4)	829 (92.2)	<0.001
Digoxin	367 (7.5)	90 (10.0)	0.009
Randomization to finerenone	2,373 (48.3)	538 (59.8)	<0.001
NT-proBNP, pg/mL	985.7 (422.5-1,890.5)	1,288.0 (633.2-2,261.8)	<0.001

Values are n (%), mean $\pm$ SD, and median (Q1-Q3), unless otherwise indicated. Among 6,001 participants in the FINEARTS-HF trial, data regarding post-baseline SBP measurements were missing in 186 participants who were excluded, leaving 5,815 participants who were analyzed in this study.

ACEI = angiotensin-converting enzyme inhibitors; ARB = angiotensin receptor blockers; ARNI = angiotensin-receptor neprilysin inhibitor; BMI = body mass index; COPD = chronic obstructive pulmonary disease; eGFR = estimated glomerular filtration rate; HF = heart failure; LVEF = left ventricular ejection fraction; NT-proBNP = N-terminal pro-B-type natriuretic peptide; SBP = systolic blood pressure; SGLT2 = sodium-glucose cotransporter 2.

following SBP <100 mm Hg, no differences were observed between study arms. The treatment effects of finerenone appeared to attenuate after hypotensive episodes, which might in part be related to the observed rates of drug discontinuation. Among participants adherent to randomized treatment, the clinical benefits of finerenone on the primary outcome appeared to be maintained, irrespective of SBP <100 mm Hg or investigator-reported hypotension occurrence (**Central Illustration**). Overall, these observations suggest that SBP drop <100 mm Hg—although prognostically relevant and increased with the nonsteroidal MRA therapy—should not necessarily deter continuation of finerenone treatment in patients with HFmrEF/HFpEF.

In patients with HFmrEF/HFpEF, hypotension is a common event with a reported incidence in clinical trials that varies based on the adopted definitions and the tested treatment. In the FINEARTS-HF study population, the incidence of SBP <100 mm Hg was similar (15% overall) to another contemporary HFmrEF/HFpEF trial—which applied a comparable definition of hypotensive events—testing the ARNI sacubitril/valsartan (13% of participants experienced post-randomization hypotension, 11% in the valsartan arms vs 16% in people randomized to sacubitril/valsartan).¹⁴ In the present analysis, the incidence of investigator-reported hypotension was approximately 6%, slightly higher than that reported in observational registries.^{15,16} However, definitions of hypotension may vary and the lack of objective SBP

TABLE 2 Independent Predictors of Post-Baseline Treatment-Emergent SBP <100 mm Hg

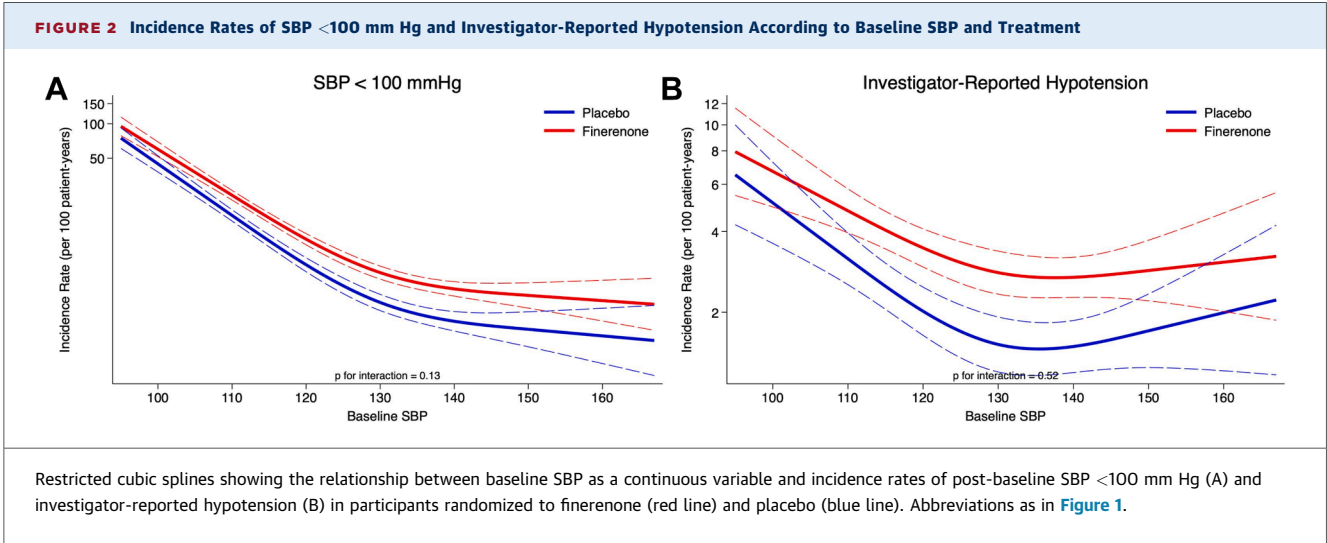
	HR (95% CI)	z	P Value
SBP (per 10 mm Hg lower values)	1.81 (1.73-1.90)	23.8	<0.001
Race			
White	Ref.		
Asian	3.01 (2.57-3.54)	13.4	<0.001
Black	1.42 (0.78-2.58)	1.1	0.25
Other	1.09 (0.71-1.66)	0.4	0.69
Randomization to finerenone	1.57 (1.37-1.80)	6.6	<0.001
Cigarette smoking			
Never	Ref.		
Former	1.42 (1.23-1.63)	4.8	<0.001
Current	1.33 (1.03-1.71)	2.2	0.028
Age (per 1 year increase)	1.02 (1.01-1.02)	4.1	<0.001
NT-proBNP (doubling)	1.10 (1.05-1.15)	3.8	<0.001
Time since HF event			
≤7 d	Ref.		
>7 d to 3 mo	1.35 (1.09-1.67)	2.7	0.007
>3 mo or no HF event	1.52 (1.23-1.86)	3.9	<0.001
SGLT2 inhibitors	1.37 (1.14-1.65)	3.4	0.001
Calcium channel blockers	0.76 (0.64-0.89)	3.3	0.001
Type 2 diabetes	0.82 (0.71-0.96)	2.5	0.011
Loop diuretics	1.31 (1.03-1.68)	2.2	0.031
BMI (per 5 kg/m ² higher values)	1.07 (1.01-1.14)	2.1	0.034

Ref. = Reference; other abbreviations as in Table 1.

measurement to corroborate investigator-reported events can limit comparisons.

In line with previous analyses, we observed that some participants' baseline characteristics were independently associated with development of SBP <100 mm Hg during follow-up, including baseline blood pressure, older age, higher NT-proBNP levels, and lack of concomitant diabetes. In contrast to what was observed with sacubitril/valsartan, the incidence of hypotension did not increase at higher LVEF values, likely due to the less pronounced vasoactive effects of finerenone. Steroidal MRA background therapy in the PARAGON-HF (Prospective Comparison of ARNI with ARB Global Outcomes in HF with Preserved Ejection Fraction) trial was associated with hypotensive events.⁴ This observation is not surprising in relation to the pharmacological properties of the MRA class that antagonizes the renin-angiotensin-aldosterone system, with suggested efficacy also for the management of resistant hypertension.^{17,18}

Within the HF population, hypotension has been shown to carry a significant prognostic impact across the full LVEF spectrum.³⁻⁵ In the present analysis, we showed that people experiencing post-baseline SBP <100 mm Hg had higher subsequent mortality



rates whereas, different from patients who have HFrEF, the risk of HF events was comparable. The greater magnitude of prognostic implications previously described in people with reduced LVEF can be explained by the underlying pathophysiologic mechanisms. In HFrEF, hypotension may reflect relative hypoperfusion due to left ventricular systolic dysfunction and low cardiac output whereas the pathophysiology of hypotensive events at higher LVEF values is more complex and multifactorial.¹⁹ The HFmrEF/HFpEF population is typically characterized by older individuals with a high burden of cardiac and noncardiac comorbidities—including atrial fibrillation and autonomic dysfunction—that require medications potentially leading to hyperpolypharmacy.^{20,21} Therefore, although the occurrence of hypotension is an important prognostic indicator, its multiple potential pathophysiological mechanisms should be taken into account when investigating and managing this heterogeneous population.

Nevertheless, our analysis confirmed that hypotensive events can potentially lead to drug discontinuation both in clinical practice and in clinical trials. Importantly, however, rates of treatment discontinuation were similar in those treated with finerenone and those with placebo. We are cautious about the interpretation of the treatment effects of finerenone subsequent to these episodes of hypotension, as this is a post-randomization assessment which may introduce biases. In the primary ITT analysis, we observed some attenuation in treatment effects after both SBP <100 mm Hg and investigator-reported hypotension, although no significant interaction was detected. We believe this may be partly explained by the significantly higher rates of study drug discontinuation following hypotensive events. Among participants who remained on study drug therapy after a hypotensive episode, the beneficial effects of finerenone appeared to persist.

In our multivariable models, background therapies, including ones with established blood pressure

TABLE 3 Clinical Outcomes Following Post-Baseline Treatment-Emergent SBP <100 mm Hg					
	No/Before SBP <100 mm Hg	After SBP <100 mm Hg	RR/HR (95% CI)	Adjusted RR/HR ^a (95% CI)	P Value
CV death and total HF events	1,929 (15.2)	347 (23.4)	1.59 (1.34–1.89)	1.15 (0.95–1.39)	0.16
Death from CV causes	402 (3.2)	73 (4.9)	1.49 (1.16–1.92)	1.37 (1.04–1.81)	0.026
Total HF events	1,529 (12.0)	274 (18.5)	1.62 (1.33–1.96)	1.10 (0.89–1.36)	0.37
HF hospitalization or urgent visit	887 (7.6)	128 (10.0)	1.50 (1.24–1.81)	1.02 (0.83–1.26)	0.83
Death from any cause	796 (6.2)	156 (10.5)	1.59 (1.34–1.89)	1.38 (1.13–1.68)	0.001
CV hospitalizations	1,439 (13.1)	175 (15.2)	1.29 (1.10–1.51)	1.03 (0.86–1.24)	0.71
All-cause hospitalizations	2,522 (26.7)	302 (35.7)	1.49 (1.32–1.68)	1.24 (1.08–1.42)	0.003

Values are n (rate per 100 py), unless otherwise indicated. ^aAdjusted for age, region, sex, randomized treatment, LVEF, SBP, heart rate, smoke, type 2 diabetes, eGFR, BMI, beta-blockers, ACEI/ARB/ARNI, SGLT2 inhibitors, atrial fibrillation, log-transformed NT-proBNP (all at baseline).
CV = cardiovascular; py = patient-years; RR = rate ratio; other abbreviations as in Table 1.

TABLE 4 Associations Between Post-Baseline Treatment-Emergent SBP <100 mm Hg, Randomized Treatment, and Clinical Outcomes

	No/Before SBP <100 mm Hg		After SBP <100 mm Hg		<i>P</i> _{Interaction}
	Placebo	Finerenone	Placebo	Finerenone	
Total HF hospitalizations and death from CV causes					0.33
Events, n	1,100	829	140	207	
Event rate, per 100 py	16.9	13.3	23.9	23.0	
Rate ratio (95% CI)	0.78 (0.67-0.90)		0.99 (0.70-1.39)		
Death from CV causes					0.26
Events, n	219	183	26	47	
Event rate, per 100 py	3.4	2.9	4.4	5.2	
HR (95% CI)	0.87 (0.71-1.06)		1.20 (0.73-1.96)		
Total HF hospitalizations					0.47
Events, n	882	647	114	160	
Event rate, per 100 py	13.6	10.4	19.5	17.8	
Rate ratio (95% CI)	0.76 (0.64-0.89)		0.93 (0.64-1.36)		
HF hospitalization or urgent visit					0.48
Events, n	503	384	52	76	
Event rate, per 100 py	8.5	6.7	10.6	9.6	
HR (95% CI)	0.77 (0.67-0.88)		0.90 (0.63-1.31)		
All-cause death					0.91
Events, n	426	370	65	91	
Event rate, per 100 py	6.5	5.9	11.1	10.1	
HR (95% CI)	0.91 (0.79-1.05)		0.94 (0.67-1.30)		
CV hospitalizations					0.67
Events, n	772	667	75	100	
Event rate, per 100 py	13.9	12.3	17.3	14.0	
HR (95% CI)	0.87 (0.78-0.96)		0.80 (0.58-1.09)		
All-cause hospitalizations					0.61
Events, n	1,324	1,198	114	188	
Event rate, per 100 py	27.8	25.5	36.1	35.5	
HR (95% CI)	0.92 (0.85-0.99)		0.99 (0.78-1.26)		

Adjusted for propensity of developing hypotension, stratified for region and LVEF.

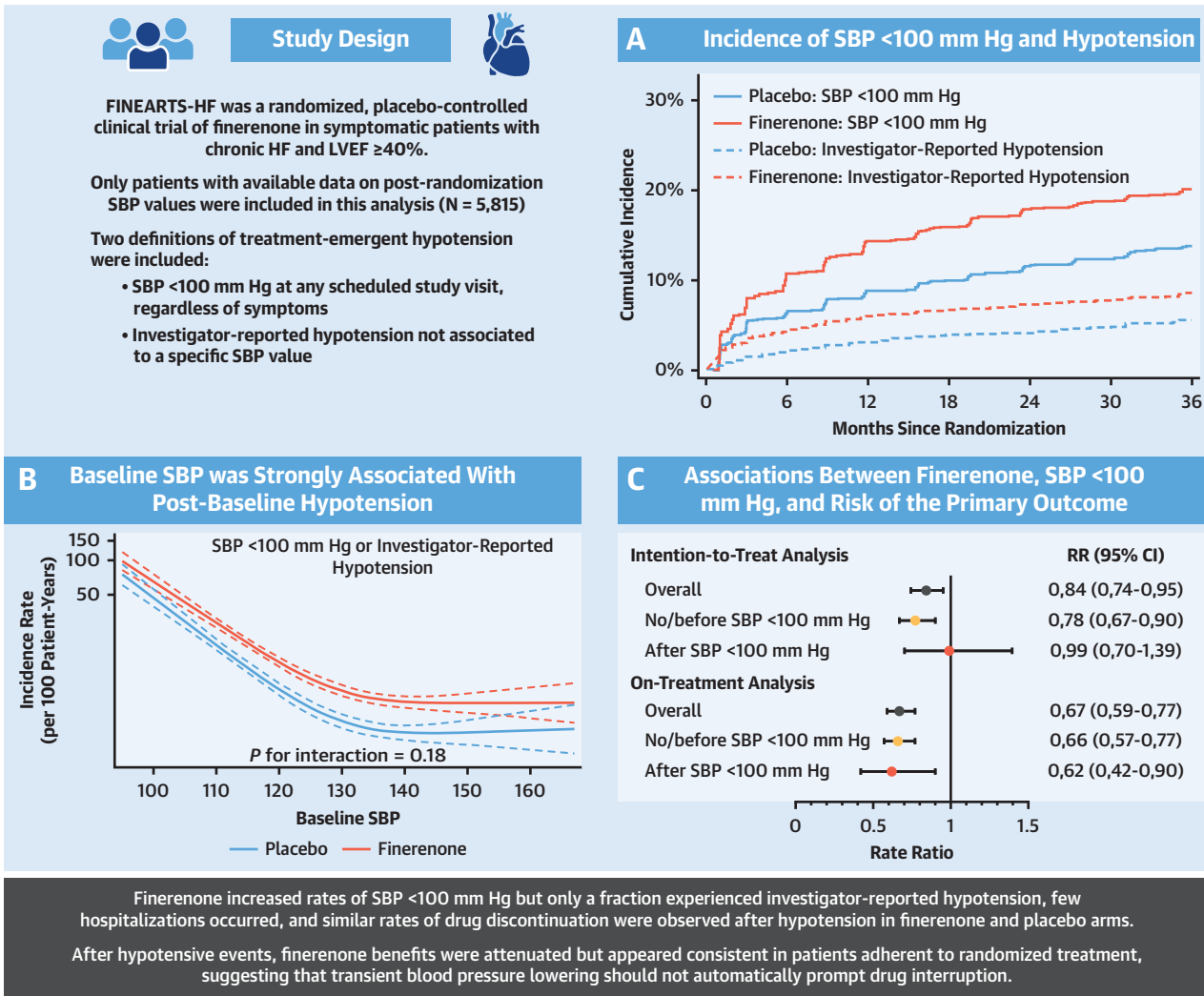
Abbreviations as in [Tables 1 and 3](#).

lowering such as sacubitril/valsartan, were not consistently associated with hypotension. However, we remain cautious in the interpretation of these data as they may be influenced by other accounted and unaccounted factors. Close follow-up is required after drug initiation especially when finerenone is started together with other hemodynamically active HF therapies. If limiting blood pressure lowering is encountered, mitigating strategies in clinical practice may include adjustment of concomitant diuretics, discontinuation of other antihypertensive drug therapies (such as calcium channel blockers), and complete review of medication lists (including drugs such as alpha blockers for the management of benign prostatic hyperplasia). Altering the timing (spacing it from other hemodynamically active therapies) or dosing of finerenone may additionally be considered to facilitate treatment continuation.

STUDY LIMITATIONS. The main analysis was performed using SBP <100 mm Hg as the definition of hypotension and based on investigator-reported

adverse events. Additionally, SBP measurements were obtained during study visits whereas other measures of blood pressure response, such as ambulatory blood pressure monitoring, were not performed. Symptoms associated with SBP <100 mm Hg or investigator-reported hypotension were not systematically collected, which may have led to under-capture of symptomatic events. The specific actions taken following hypotensive events, including potential changes in background medications such as diuretics, were similarly not systematically collected. Study drug discontinuation was evaluated based on timing from hypotension and, therefore, causality cannot be assessed. Additionally, temporary discontinuations in relation to SBP <100 mm Hg occurrence were not systematically collected. Hypotension occurred post-randomization and, despite propensity score-adjustment, estimates of treatment effects following hypotension events may remain residually confounded. Lastly, data regarding post-baseline SBP measurements were missing from 186 participants

CENTRAL ILLUSTRATION Finerenone-Related Risk of Hypotension in HFmrEF/HFpEF: the FINEARTS-HF Trial



Foà A, et al. JACC Heart Fail. 2026;14(4):102779.

(A) Cumulative incidence of systolic blood pressure (SBP) <100 mm Hg (solid lines) and investigator-reported hypotension (dashed lines) in participants randomized to finerenone (red lines) and placebo (blue lines). (B) Restricted cubic splines showing the relationship between baseline SBP as a continuous variable and incidence rates of the combined outcome of post-baseline SBP <100 mm Hg or investigator-reported hypotension in participants randomized to finerenone (red line) and placebo (blue line). (C) Associations between finerenone, SBP <100 mm Hg, and the risk of the primary outcomes. The forest plot shows finerenone-related effects on the primary outcome based on SBP <100 mm Hg in both intention-to-treat and on-treatment analyses. FINEARTS-HF = Study to Evaluate the Efficacy (Effect on Disease) and Safety of Finerenone in Participants With Heart Failure and Left Ventricular Ejection Fraction (Proportion of Blood Expelled Per Heart Stroke); HF = heart failure; HFmrEF = heart failure with mildly reduced ejection fraction; HFpEF = heart failure with preserved ejection fraction; LVEF = left ventricular ejection fraction; RR = rate ratio.

who were excluded from the analysis, potentially leading to some degree of selection bias.

CONCLUSIONS

In this prespecified analysis of the FINEARTS-HF trial, we found that finerenone increased rates of

SBP <100 mm Hg. However, only a fraction of hypotension was reported by investigators as adverse events, few hospitalizations related to hypotension occurred, and similar rates of drug discontinuation were observed after hypotensive events in finerenone and placebo arms. Following hypotension, finerenone benefits were attenuated but appeared consistent

among participants who were adherent to randomized treatment, suggesting that transient blood pressure lowering should not automatically prompt drug interruption. These data may inform anticipatory guidance to clinicians and patients when initiating and continuing finerenone in clinical practice.

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The FINEARTS-HF trial was sponsored by Bayer AG. Dr Vaduganathan has received research grant support, served on advisory boards, or had speaker engagements with Alnylam Pharmaceuticals, American Regent, Amgen, AstraZeneca, Bayer AG, Baxter Healthcare, Bristol Myers Squibb, Boehringer Ingelheim, Chiesi, Cytokinetics, Esperion, Fresenius Medical Care, Idorsia Pharmaceuticals, Lexicon Pharmaceuticals, Merck, Milestone Pharmaceuticals, Novartis, Novo Nordisk, Pharmacosmos, Recordati, Relypsa, Roche Diagnostics, Sanofi, and Tricog Health; and has participated on clinical trial committees for studies sponsored by Amgen, AstraZeneca, Boehringer Ingelheim, Galmed, Novartis, Bayer AG, Occlutech, Pharmacosmos, and Impulse Dynamics. Dr Claggett has received consulting fees from Alnylam Statistical, Cardior, Cytokinetics, CVRx, Intellia, Rocket, and Eli Lilly outside of the submitted work. Dr Desai has received grants from Bayer; an institutional research grant to Brigham Women's Hospital to support clinical endpoints committee activities; consulting fees from Bayer during the conduct of the study; institutional grants from Abbott, Alnylam, AstraZeneca, and Novartis; and consulting fees from Avidity Biopharma, Axon Therapeutics, Biofourmis, Boston Scientific, Endotronix, GlaxoSmithKline, Merck, Medpace, Icon Clinical Research, Medtronic, New Amsterdam, Novartis, Abbott, Alnylam, AstraZeneca, Parexel, Regeneron, Roche, River2Renal, scPharmaceuticals, Veristat, Verily, and Zydus outside of the submitted work. Dr Jhund has received research support to his institution from Bayer AG, during the conduct of the study; research support from AstraZeneca; speaker and advisory board fees and research support to his institution from NovoNordisk; grants from AstraZeneca, Analog Devices Inc, ProAdWise Communications, and Roche Diagnostics outside of the submitted work; and has served as the director of GCTP Ltd. Dr Henderson has received travel and consulting fees from Bayer outside of the submitted work. Dr Chatur has been supported by the American Heart Association Career Development Award and the Massachusetts General Hospital Spark Award; and has received grant funding, speaking honoraria, or consultancy fees from Bayer, Roche, and Us2.ai. Dr Amarante is a full-time employee of Bayer SA. Dr Hofmeister is an employee of Bayer AG. Dr Horvat-Broecker and Ms Kuhls are employees of Bayer AG. Dr Lam has received grants from the National Medical Research Council of Singapore, NovoNordisk, and Roche; consulting fees from or been on advisory boards, steering committees, or executive committees with Alleviant Medical, Allysta Pharma, Alnylam, AnaCardio, Applied Therapeutics, AstraZeneca, Bayer, Biopeutics, Boehringer Ingelheim, Boston Scientific, Bristol Myers Squibb, CardioRenal, Corteria, CPC Clinical Research, Cytokinetics, Eli Lilly and Co, Impulse Dynamics, Ionis, Intellia Therapeutics, Janssen, Medscape/WebMD, Merck, Novartis, NovoNordisk, Prosciento, Quidel Corp, Radcliffe Group Ltd, Recardio, ReCor Medical, Roche, Sanofi, Siemens Healthcare Diagnostics, and Us2.ai; is cofounder and nonexecutive director of Us2.ai; has a patent pending for PCT/SG2016/050217; and has U.S. patent 10,702,247 issued outside of the submitted work. Dr Senni has received personal fees from Novartis, Bayer, Merck, Merck Sharp & Dohme, Abbott, NovoNordisk, AstraZeneca, Boehringer Ingelheim, Cardurion, Amgen, and Vifor outside of the submitted work. Dr Shah has received consulting fees from Bayer during the conduct of the study; grants from the National Institutes of Health, American Heart Association, AstraZeneca,

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PERSPECTIVES

COMPETENCY IN MEDICAL KNOWLEDGE: In HFmrEF/HFpEF patients treated with finerenone, the occurrence of SBP <100 mm Hg is common whereas hypotension reported as an adverse event is less frequent, rarely leading to hospitalizations. Among patients who were adherent to treatment, clinical benefits of finerenone appear consistent irrespective of the occurrence of hypotension.

TRANSLATIONAL OUTLOOK: The pathophysiology of blood pressure lowering and hypotension in patients with HFmrEF/HFpEF is multifactorial as these people typically have several comorbidities often requiring polypharmacy. In patients treated with finerenone, the occurrence of hypotensive events should not automatically prompt drug interruption as people may still derive long-term clinical benefits.

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KEY WORDS finerenone, heart failure, hypotension, systolic blood pressure

APPENDIX For supplemental tables, please see the online version of this paper.